

100 YEARS OF THE HEALTHY DEBATE INITIATIVE: REVISITING WALTER SWIFT'S "THE HYGIENE OF THE VOICE BEFORE DEBATES"

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Abstract: Dr. Walter Swift published the essay, "The Hygiene of the Voice before Debates" in the second volume of the *Quarterly Journal of Speech Education*. As a physician, Dr. Swift had noticed that the practice of intercollegiate debate had tremendous intellectual and social rewards, but worried that the debaters were sacrificing too much in the name of competition. He stressed the importance of diet, rest, and a variety of other initiatives designed to improve the health of the debaters involved in the rigors of intercollegiate debate. This essay will examine how the intercollegiate debate community continues to struggle with the balance between the demands of competition and the health of the participants. In the century since Dr. Swift's essay, medical research has demonstrated that many of his claims were correct. This essay, however, will expand the scope beyond the debaters to examine the intercollegiate practices that put our debaters, coaches, and directors at risk of burnout and/or long term health problems. The fact that Dr. Swift's essay is still relevant nearly one-hundred years later suggests that there are no easy solutions, but ignoring the topic (especially in light of the medical research) does a disservice to everyone involved in intercollegiate debate.

From June 2009 through July 2010 the NDT/CEDA policy debate community experienced a series of tragic deaths that shook us to the core. This is not to say that there were not tragedies before or that there have not been tragedies after this period, but instead it is to suggest that the passing of our close

friends and mentors Frank Harrison then John Gossett then Ross Smith then Scott Deatherage and then Mark Deloach in such a short period served as a wakeup call to those of us that had the privilege of knowing them. There is too little space for any one essay to review their accomplishments and to do justice to their legacies. Suffice it to say: these men touched the lives of countless students and in the process made the debate community a better place. The proximity of the tragedies to each other made it difficult for the debate community to articulate an appropriate response to each death and even more difficult to find the perspective necessary to put these men's lives into focus and begin the healing process. The simple refrain that was repeated over and over again was that they had contributed so much and passed away too soon.

As the shock of their deaths passed over us, a conversation began and centered on what we as a community could do to help improve the future for those of us that have decided to build careers as forensic educators in intercollegiate debate. As one can imagine, a group of debate educators "discussing" the future of the activity produced a myriad of responses ranging from initiatives to de-emphasize competition to arguments about the inevitability of health related concerns for anyone that spends many weekends in a tournament setting. Although there was little consensus about which proposals had the greatest potential for meaningful change, there was little disagreement that *something* needed to be done. After all, we had just lost some of the most important voices that had been at the forefront of helping our community change for the better. Now it was our turn to figure out a way forward in hopes of helping each other be a part of the community we all love for as long as possible.

While discussions were taking place in many

settings and across a variety of platforms, Sherry Hall helped direct the conversation by announcing the start of a Healthy Debate Initiative. The goal was simple: research and advocate for changes to the practices of intercollegiate debate to promote health. Everyone invested in the Healthy Debate Initiative acknowledged upfront that it was unlikely that the initiative could do much to influence the choices that debate educators make when they are not at tournaments. Additionally, there will always be a group of coaches that focus exclusively on competition over everything else. That being said, everyone agreed that tournaments can be stumbling blocks that make it more difficult to sustain a healthy lifestyle inside and outside of debate. The goal, therefore, was to help improve the tournament experiences for the debate educators and students that do want to prioritize making healthy decisions while still participating in intercollegiate debate.

As is the case with most research projects, the people invested in the Healthy Debate Initiative discovered that they were not the first forensic educators worried about the health implications of intercollegiate debate. In fact, one of the first articles written in *The Quarterly Journal of Public Speaking*, now *The Quarterly Journal of Speech*, was a 1915 essay by a medical doctor, Dr. Walter Swift, entitled, “The Hygiene of the Voice before Debates.” In this essay, Dr. Swift reviewed the contemporary medical science of his day in an attempt to convince intercollegiate debaters to make healthier choices in the name of competition. Dr. Swift targeted his advice exclusively for the debaters and was writing at a time before tournaments became the norm for competition. We have found his basic goal of using medical science to persuade participants to make healthier choices persuasive. Our goals in this essay, therefore, are to examine and extend Swift’s

claims in light of contemporary medical science, to make a series of recommendations to tournament directors in hopes of promoting healthier tournament settings for all participants, and to suggest areas for future research.

Revisiting Swift: Updating the Science of Vocal Hygiene

One-hundred years ago Dr. Swift initiated a conversation over the relationship between making healthy choices and success in competitive debate. He acknowledged at the outset of his essay that as a physician his medical knowledge was rarely viewed as a resource for debate coaching advice, but that he hoped that debaters would pay attention because, “Now I, as a physician, am very interested in something without which you cannot debate. . . That something is the voice” (Swift, 1915, p. 115). Although this introduction was designed to grab the attention of his audience of intercollegiate debaters by focusing on the instrument of their argumentation, Dr. Swift went on to define “hygiene” broadly to mean “the principles of health” (1915, p.116). The key distinction for Dr. Swift was that hygiene was less about a doctor’s intervention to stop an illness and more about keeping people in good “health and making them even more healthy than they are” (1915, p. 116).

We agree with Dr. Swift’s call for a broader assessment of the practices of intercollegiate debate and how they contribute to the overall prospects for living a healthy life while participating in intercollegiate debate as directors, coaches, assistant coaches, and debaters. A cursory glance reveals that Dr. Swift relied on medical science that is antiquated, but that his broader themes are still relevant for the modern intercollegiate debate experience. In particular, we will examine Dr. Swift’s

discussions of exercise, sleep, and diet. There are some additional sections of Dr. Swift's essay that are interesting (and still relevant), including an in-depth defense of the need for debaters to bathe more often, but we believe that researching exercise, sleep, and diet are closer to the goals of the Healthy Debate Initiative which focuses on using the tournament experience to support the healthy decisions people are hopefully making at home. As is the case with any discussion of health related concerns, there is quite a bit of overlap which makes it difficult to address any one health-related concern in isolation. Our hope is to make a persuasive case for a set of recommendations for each concern independently, but with a simultaneous call for tournament directors to address these issues as holistically as possible.

Exercise

Exercise was a key component of Dr. Swift's recommendation for debaters. He argued that proper exercise is necessary because "It keeps the blood in circulation, it enables the blood to carry proper nutrition to all parts of the body, and it maintains the muscles in proper form and development" (1915, p. 117). Although he does not include much additional medical science, he does go to great lengths to explain that exercise programs should be created based on the individual needs of the person such as his advice that "Those who are confined in bad air and use their arms a good deal should exercise themselves by walking" (1915, p. 118). Although that advice makes intuitive sense, we have the luxury today of newer studies that confirm that exercise is, in fact, a crucial component to any discussion of promoting healthy decisions including some studies that are applicable to the tournament setting.

Studies confirm the overarching claim that exercise promotes general wellbeing including lower rates of chronic diseases and higher levels of fitness (Physical Activity Guidelines Advisory Committee, 2008). In addition to adults, research specific to college-aged students confirms that exercise is a crucial component of overall wellbeing (Greene, et al., 2012; Gruber, 2008; Penedo and Dahn, 2005). Beyond general wellbeing, studies confirm that physical activity and exercise reduce obesity, diabetes, and blood pressure (Andersen et al., 2006; Bell, et al., 2007; Huang et al., 2003). Additionally, physical activity decreases psychological stress and depression (Cai, 2000; Norris, Carroll, and Cochrane, 1992). Finally, physical activity increases cognitive functioning (Etnier, Nowell, Landers, and Sibley, 2006; Kramer and Erickson, 2007; Tomporowski, 2003). While overall wellbeing is certainly important, the other variables: obesity, diabetes, blood pressure, stress, depression, and cognitive functioning were all mentioned in the community discussions of the health problems surrounding intercollegiate debate. These community discussions are occurring in a variety of spaces including social media and online forums. The average tournament experience involves at least seventy-two hours of professional work that has the potential to exacerbate these negative health outcomes. Developing opportunities for exercise and physical activity is an important way to counterbalance some of the stumbling blocks presented by tournaments.

Studies have found that even small amounts of moderate-to-vigorous physical activity improves health outcomes (Janssen and LeBlanc, 2010). Despite the fact that people can benefit from smaller amounts of exercise, sustaining any exercise routine in a tournament setting can be very difficult. There are a variety of reasons why this is the case including, but not

limited to; the rigorous demands of the schedule, the difficulty in finding an appropriate space to exercise, and the overall feeling of exhaustion after a full day of activities. Dr. Swift emphasized the importance of exercise, but his recommendations were outdated while also demonstrating the difficulty of incorporating exercise into the tournament experience. For instance, he suggested, “One hour a day of pretty severe exercise, like gymnasium work or a long walk or an hour’s row on the water, taken about the middle of the day or in the late afternoon, is sufficient to keep the average man in good muscular trim” (1915, p. 118). In an effort to bring the medical science to a set of more manageable set of tournament practices we offer three tournament recommendations to improve access to exercise.

Recommendation #1: Tournament hosts should offer a walking tour of campus during a designated debate. Many community members note that they visit college campuses throughout the country, but rarely see more than the handful of buildings used by the tournament. A walking tour offers judges and coaches an opportunity to step outside of the competitive landscape to enjoy a campus while engaging in some basic physical activity. According to the Physical Activity Guidelines Advisory Committee, “Strong evidence shows that a regimen of brisk walking provides a number of health and fitness benefits for adults and older adults, including lower risk of all-cause mortality, cardiovascular disease, and type 2 diabetes” (2008, n.p.). This may be particularly helpful for debate educators given that the committee also found that, “Strong evidence also shows that frequent bouts of walking increase cardiorespiratory and metabolic fitness, especially in people who have been performing little activity on a regular basis” (2008, n.p.). It may be possible for some tournaments to incentivize the

walking tour by offering to hire replacement judging to cover the commitment of any judge that chooses to participate.

Recommendation #2: One of the debate organizations should sponsor an intramural sports league with competitions during tournaments. Although the topics change from year to year, many of the same coaches find themselves interacting with each other from tournament to tournament across decades. We believe that it may be possible to draw upon the competitive impulses of the participants to create an intramural league for a variety of sports that can be played across college campuses with little equipment requirements including, but not limited to, tennis, basketball, soccer, and ultimate Frisbee. The debate organization would be responsible for designing the schedule, recruiting participants, and awarding prizes. Similar to our first recommendation, tournaments may need to coordinate with the intramural league officials to ensure the participants have the appropriate round off and have access to on-campus facilities.

Recommendation #3: Tournament hosts should negotiate hotel contracts that ensure access to a gym including negotiating for additional hours for hotel gyms and/or prioritize hotels that have twenty-four hour access to the gym. One of the persistent problems identified by debate community participants is the time constraints that stem from the tournament schedule. Many tournaments release pairings by 7:00am which requires participants to be up and prepared to depart for the tournament before many hotel gyms are open for use. Participants that wait until after the day of debates often return to the hotel to find the gym facilities are closed for the day. Tournament hosts should negotiate a reasonable extension for the morning and the evening based on the schedule

provided to ensure access if participants would like to take advantage of the hotel facilities.

Although there is no replacement for regular exercise at home, our hope is that these recommendations can help reduce the current description of debate tournaments wherein folks move from room to room to sit and evaluate debates in three hour increments making it possible to spend an entire day without engaging in much if any physical activity. Our next challenge is to help provide participants with enough sleep to encourage them to engage in the opportunities for physical activity.

Sleep

Although many attitudes about debate have changed since 1915, Dr. Swift's assessment of how debate participants view sleep rings true today, "Well people usually consider rest absolutely unnecessary" (p. 118). Studies confirm that far too often adults and college students do not get adequate sleep (Bublitz et al., 2009; National Sleep Foundation, 2004; Orzech, Salafsky, and Hamilton, 2011). The anecdotal evidence from intercollegiate debate participants suggests that debate tournaments are particularly poor at creating opportunities for adequate rest and the implications of inadequate sleep are troublesome. There are, of course, immediate safety concerns such as long drives after multiple days of inadequate sleep, but there are also long term health implications that stem from not prioritizing adequate sleep. According to Dr. Swift, people with inadequate sleep "...show some form of nervousness, like indigestion, or constipation, or some other minor defect that goes to prove that their lack of rest and change of work is to blame for their ill health" (1915, p. 119). Although he does argue that "...systematic rest is one of the most important means

of maintaining a normal hygiene," (1915, p.119) the updated medical science demonstrates that there are far graver consequences to inadequate sleep than indigestion or constipation.

Similar to exercise, studies have found links between inadequate sleep and a variety of general health benefits (Moore and Adler, 2002; Walsh, Dement, and Dinges, 2005). In some ways, adequate sleep is the gatekeeper to exercise and diet as people's decision making abilities diminish with inadequate sleep. According to several studies, there is a link between adequate sleep and cognitive performance for adults and college students in particular (Gaultney, 2010; Horne, 1988; Killgore, 2010; Medeiros, Mendes, Lima, and Araujo, 2001; Pilcher and Walters, 1997; and Trockel, Barnes, and Egget, 2000). Additionally, adequate sleep is an important way to improve psychological health and reduce stress (Gray and Watson, 2002). In the long term, adequate sleep is essential for adults that make a career out of being a forensic educator as studies indicate a link between adequate sleep and cognitive performance for older adults (Cricco, Simonsick, and Foley, 2001). Given the importance of cognitive performance for many of the roles required by a forensic educator including driving, coaching, and judging, adequate sleep is an essential component of any long term career in intercollegiate debate. Tournament settings must confront the need to provide opportunities for adequate sleep for the people with the full knowledge that some coaches and debaters will elect not to use them.

The primary obstacle to providing adequate sleep at tournaments is the tension between creating a schedule that prioritizes a good night's rest and maximizing opportunities for debates. As debate budgets have shrunk, program administrators have attempted to

find the most efficient ways to provide opportunities for debates which incentivizes debate programs to choose tournaments that provide the most debates. The problem is that unless there is a substantial change to the format and judging norms, a day of debates requires a tremendous amount of time. Tournaments must budget a minimum of two hours and thirty minutes for a debate to occur and be decided, debaters expect an additional thirty minutes of post-round discussion with the judge, and coaches expect a minimum of thirty minutes of preparation time before the debate. That means each debate requires a minimum of three hours and thirty minutes. In order for a tournament to have four rounds in a single day (as required by any tournament with eight preliminary rounds) the debating alone requires fourteen hours. Despite the length of time required to conduct four debates, many coaches and debaters have expressed frustration at the tournaments that use this truncated time formula and argue that judges should be allotted more time for deciding debates and that debaters should have a minimum of forty-five minutes for preparation. A competition day could easily extend to sixteen hours if tournaments agree to the pressures of competition and extend the times allotted for judges and pre-round preparation.

Even with the fourteen hour day schedule, the tournament must allocate time for meals, accumulate results, pair the rounds, and give the debaters adequate time to move between rooms. Assuming maximum efficiency, a tournament is likely to begin at 7:00am and conclude at 9:00pm. Everyone involved in forensics knows that maximum efficiency is a near universal goal, but that very few tournaments have the staff, resources, and know-how to consistently achieve it. Instead, tournaments often run late which extends a fourteen

hour day into an even longer day.

Days designated for elimination rounds are notoriously difficult to navigate. The day often starts with the double-octofinals which means that there are five rounds in one day to crown a champion. Elimination days are also much longer as the debaters and coaches expect more preparation time while judges expect more time to make their decisions. This all happens at the conclusion of the tournament which means that everyone is already tired from two previous days of rigorous competition. It is not uncommon for an elimination day to begin at 7:00am and conclude after 1:00am. The most common excuse is that the small number of people involved in the final round are so excited to be there that they are happy to sacrifice their sleep for the chance to win the tournament. That argument is, of course, precisely what Dr. Swift saw when he stated in 1915 that "...people usually consider rest absolutely unnecessary" (p.118).

How can our community provide adequate opportunities for sleep if we are so focused on competition that we construct schedules that require fourteen hour days (assuming everything runs perfectly)? There is no simple solution and we understand that there is little prospect for challenging the norms for eight preliminary rounds, expanded preparation time, and increased demands for judges to decide debates. Our suggestions, therefore, are attempts to work within existing constraints given the power of the norms involved and the push for more debates at each tournament to maximize debate budgets. At the individual level, we recommend that coaches and debaters put a strong emphasis on finding time for adequate sleep at home because there is little prospect of significant gains in sleep in a tournament setting. In terms of the tournament setting, we believe there

are three things that tournament directors could do to improve the opportunities for coaches to get more sleep.

Recommendation #1: Tournaments should provide dinner to reduce the time obligations required after the rounds are complete. If a tournament runs perfectly and everyone leaves campus at 9:00pm there is an opportunity for some coaches to choose to get a decent night's rest if they can go directly to the hotel and sleep. If a tournament does not provide dinner then the night extends further as teams venture out to dinner which forces everyone to choose between faster/unhealthier food options and getting more sleep. If a tournament cannot provide dinner then the schedule should be designed to ensure that the participants either have a long enough break to secure food on their own between the third and fourth debate of the day or eliminate the fourth round from each day altogether.

Recommendation #2: Tournaments should provide spaces for participants to rest. Quiet and/or dark rooms are an important component of ensuring that a tournament setting can include a place of rest for people that want and/or need rest during the tournament. These spaces should be designated in advance and publicized to all participants. It may be difficult for the debaters to take advantage of the space given the hectic demands of the schedule, but the spaces should be available to them and any coach that chooses to prioritize rest during the day.

Recommendation #3: In order to facilitate more rest for coaches, tournaments should reduce judging requirements for coaches based on the length of their service to the community. We do not have a particular formula to propose, but we believe that as a community we should care about preserving the long term viability

of coaching intercollegiate debate. One way to do that is to reward long term service to the community. If a coach does not want the reduced commitment then he/she can always elect to judge more debates. If a coach that has served our community for a number of years then they could be rewarded with an opportunity to get meaningful rest during each day of the tournament then we believe it is a worthwhile investment. Alternatively, the tournament could prioritize giving rounds off to coaches that have a certain set of responsibilities for their programs. An example would be allowing programs to designate a driver that would receive rounds off if the tournament has surplus judging.

We understand that these solutions will not correct the fundamental sleep problem that exists in modern policy debate community. As coaches we want to reward the hard work of our debaters by providing them as many opportunities to debate as possible which requires tournament formats that sacrifice sleep in the name of competition. In our estimation, norms have continued to develop in ways that promote more and better competition, but that comes at a cost as we stretch schedules further and further to meet the time demands required. Without the power of an organizational response, such as a drastic amendment that would only sanction tournaments that had six or less preliminary debates or one that set a maximum number of hours allowed per day, we do not see many opportunities for adequate sleep. A drastic measure may be necessary to access the true health benefits of adequate sleep for student cognition. For example, studies support the conclusion that delaying the start of school can help promote better academic performance (Gruber and Raviv, 2003; Onyper, Thacer, Gilbert, and Gradess, 2012; Owens, Belon, and Moss, 2010). The only way to access those educational benefits, however,

would require moving away from the four rounds a day model which is not the current trend in debate. Next we turn to Dr. Swift's third major recommendation which we believe tournament directors have far more control over—the food provided at tournaments.

Diet and Nutrition

We appreciate that one-hundred years ago, Dr. Swift was concerned about the diet and nutrition of debaters. That being said, this is the portion of his essay where modern science has much to contribute to Dr. Swift's discussion of overall health. Much of his advice centered on the process of eating. He was very concerned, for instance, on proper chewing techniques. He wrote, "...proper chewing of the food...is now recognized medically as one of the most important procedures in eating; it enables the gastric juices and the intestinal digestive juices to penetrate and dissolve our food and thereby it increases absorption" (1915, p. 120). Additionally, Dr. Swift argued "...we should not take too many liquids" and "...we should not take at one meal several indigestible substances together" (1915, p. 119). Despite his emphasis on technique, it is clear that Dr. Swift knew that diet and nutrition are important components of health and wellbeing.

More recent medical studies support the role that tournament directors play in helping shape the health of all the participants at a tournament from the debaters to the coaches. To begin, studies have found that adequate nutrition is important to cognitive functioning (Bellisle, 2004; Black, 2003; Santos, 2008; Taras, 2009). The rigorous demands of the modern tournament require more attention to nutrition given the expectations we all have for our debaters' cognitive functioning in the heat of battle. More importantly, tournament directors have a chance to help people

make better choices outside of the tournament setting as studies have found that providing healthy food in catering situations can improve overall nutrition and eating behaviors beyond the immediate meal (Raulio, Roos, and Prattala, 2010). For young people in particular, eating patterns in college are uniquely important for lifelong health (Deshpande, Basil, and Basil, 2009). Additionally, studies confirm that caterers should not simply provide inexpensive and/or convenient options, but should emphasize health in food provision (Burns and Gregory, 2007). The culmination of this research suggests that tournament directors have an opportunity to go beyond simply providing food to helping make a difference in the success of the debaters and the overall wellbeing of all of the participants.

Although we will offer a series of recommendations below, we want to acknowledge that on balance tournaments have made dramatic improvements in their food offerings. Not that long ago, breakfast consisted of donuts/bagels, lunch consisted of pizza, and dinner was fast food on the way home after a long day of debates. Thanks, in part, to the Healthy Debate Initiative more tournaments are providing healthier options for meals while simultaneously expanding the number of meals provided by the tournaments. The Harvard tournament, for example, prioritizes healthier high protein snacks between rounds to boost energy for the participants. The Wake Forest tournament now provides fruit and Greek yogurt as alternatives to the donut and bagel breakfast items of the past. Some tournaments have already implemented some of the recommendations below. Our goal here is to help promote some of the best practices we have seen in recent years.

Recommendation #1: Tournaments should

provide meals during regular mealtimes with high protein snacks in between meals. Studies support the importance of timing as a key variable in health and obesity (Garaulet et al., 2013; Karnarek, 1997; and Mattes and Campbell, 2009). Timing can be difficult given the extreme schedule, but more tournaments are offering lunch between rounds one and two then dinner between rounds three and four. That schedule works as long as there are plenty of high protein snacks available throughout the day to supplement the meal times. Many coaches have remarked that they are most likely to stumble at a tournament without a regular eating schedule and/or the availability of healthy snacks. In these situations, people tend to turn to vending machines or fast food which makes subsequent healthy eating decisions more difficult.

Recommendation #2: Tournaments should take breakfast seriously and provide nutritious options. There are a number of studies that support the conclusion that skipping breakfast undermines health (Dubois et al., 2009; Keski-Rahkonen et al., 2003; Rampersaud et al., 2005; Smith et al., 2010). In particular, skipping breakfast undermines cognitive functioning and academic performance (Simeon and Grantham-McGregor, 1989). As tournament directors, we know that investing in breakfast can be costly and that it can be frustrating to provide a full breakfast only to have participants take coffee and a granola bar. Although it may be frustrating, a robust breakfast is essential for participants that eat breakfast regularly and the lack of options is a consistent excuse for why participants and coaches do not emphasize it more. This is a norm that we believe can change if tournament directors and coaches emphasize breakfast for themselves and their debaters. We acknowledge, for example, that the West Georgia tournament was

known for providing a robust hot breakfast for their participants.

Recommendation #3: Tournament directors, coaches, debaters, and anyone involved in a tournament setting should communicate all food allergies to tournament hosts. As tournaments provide more and better food options, coaches and debaters cannot rely solely on their own abilities to identify food ingredients. Tournament directors should continue to request any allergy information and consider being proactive by noting any food options that contain ingredients that are common food allergies such as peanuts. Communicating information about food allergies is an essential component of ensuring that the tournament setting is a safe and healthy place for all participants.

In many ways, diet and nutrition is one of the most difficult things to change at a tournament because of the cultural associations that go with food. Many tournaments provide “local” food as an act of hospitality designed to provide participants with a flavor of the city that they are not likely to experience otherwise. This, of course, is a noble enterprise and we certainly support the best hospitality possible, but the anecdotal evidence suggests that participants need healthy options. Many coaches have described attempting to change their diet and nutrition at home only to find themselves devouring donuts at 6:30am because they were hungry and it was the only option. That can set in motion a day’s worth of poor food decisions which are compounded by a lack of sleep and exercise.

Areas for Future Research

Dr. Swift’s essay is a fascinating window into what debate was like in 1915. We doubt that he would recognize much of what debate is today, but

his discussions of health demonstrate that our activity has long struggled with balancing the competitive demands of rigorous debate with the basic needs required to sustain healthy living. We have no doubt that if someone stumbles onto this article one-hundred years from today that they would find parts of it as amusing as we find Dr. Swift's detailed advice for how to take a bath, "Remain for fifteen to twenty minutes in really hot water, then retire at once without getting chilled, preferably in a warm room and a warm bed, the windows being opened afterward" (1915, p.121). That being said, we hope that the readers will understand that what we do share with Dr. Swift is an intense concern for everyone involved in college forensics regardless of their position or the format of their activity. Towards that end, we would like to suggest three additional areas for future research.

First, we suggest that researchers help collect more data on the experiences of forensic educators and debaters. As we were preparing this essay we were able to access substantial anecdotal data that suggests intercollegiate policy debate participants struggle with exercise, sleep, and diet. We do not, however, have a robust longitudinal data set that can help us distinguish trends, compare populations, or make more nuanced recommendations. We understand that gathering this type of data would be difficult and add yet another demand on an already overstretched set of obligations for people that have made a career out of participating in debate. That being said, we need this type of data to make better arguments to our administrations about supporting debate. As budgets are continually cut, we believe people are doing the best with what they have, but that healthy decisions often require resources and those are severely lacking at the moment.

Second, we suggest that researchers help generate

a set of best practices to help with immediate safety concerns. Although not the focus of this article, we are confident that the debate community is not the first group of people that have had to struggle with basic safety questions such as how long is it safe for one person to drive? Or, how many hours of sleep should a person have before making a long drive? Some universities have guidelines, but we believe we would all benefit from a set of best practices that could be used to develop new policies for tournaments. For instance, should tournaments require judges to be available one round past their team's performance on elimination day? Many coaches ask to be relieved so that they can begin the long trek back home. There is a great deal of subjectivity when tournament directors decide in the moment whether a six hour drive merits being relieved of the obligation to judge. We need a set of data to help create a set of policies that tournament directors could announce in advance rather than the current system which seems ad hoc at best.

Third, we suggest that researchers use the conversation over healthy debate practices as a stepping stone to examining other health issues that are beyond the scope of tournament directors. Substance abuse, for example, is beyond the scope of this essay, but is an extremely important topic for any discussion of healthy communities. We hope that research into healthy debate practices could move beyond the fear that asking tough questions will somehow put the community in jeopardy and instead focus on the health of our students and colleagues. Researches could help determine whether substance abuse rates are higher in the debate community? Researches could help generate potential health interventions that are uniquely suited for the debate community to combat substance abuse. In short, research into helping create healthier debate

tournaments could lead to a series of questions that move towards a community that cares about helping our colleagues and debaters that is much broader than the scope of this essay.

We are confident that the brilliant people involved in intercollegiate debate can approach these questions with the care and dedication they deserve. We believe that we must work together to establish an ethos of caring for each other in hopes of preventing another wake-up call that again leaves us murmuring that they gave so much and left us too soon.

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